

GOVERNMENT OF INDIA | EX-SERVICEMEN CONTRIBUTORY HEALTH SCHEME

ECHS DIRECTORATE | FRAUD ANALYTICS & FINANCIAL INTELLIGENCE REPORT

POLICY ABUSE & ENTITLEMENT MISUSE

ECHS FRAUD ANALYTICS

MODULE 19 REPORT | DATA SCOPE: FY 2021 – FY 2026

CLAIMS SCANNED	ANOMALIES DETECTED	FRAUD PATTERNS	ROOM UPGRADE FRAUDS	MAX DEDUCTION RATE
2.31 Crore	19.4 Lakh+	10	42,150	86.93%

IIT KANPUR — Data Analytics & Fraud Intelligence Division

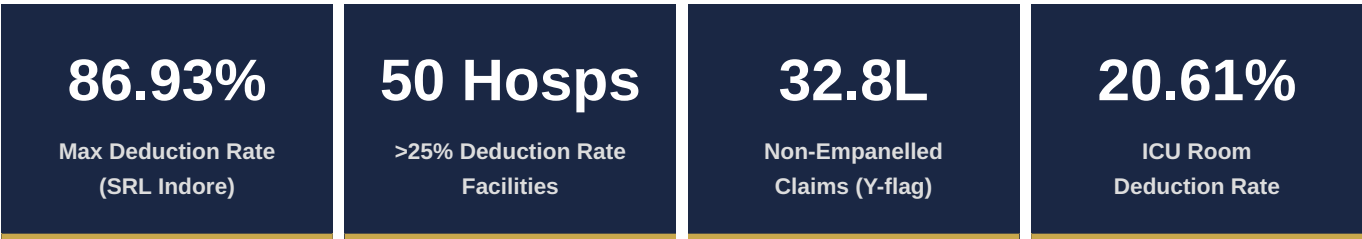
June 2026 | Ex-Servicemen Contributory Health Scheme (ECHS)

EXECUTIVE SUMMARY

Module 19 investigates three dimensions of policy abuse within the ECHS claim ecosystem: **(1)** hospitals with chronically elevated deduction rates indicating systematic overbilling; **(2)** room entitlement misuse where patients are billed for higher-category rooms than their ECHS card authorises; and **(3)** non-empanelled hospital claims where beneficiaries seek treatment at facilities not on the approved ECHS panel.

Analysis spans **43.5 million claim records** across the full ECHS database. The most alarming finding is the concentration of extreme deduction rates (>86%) in diagnostic labs and specialist facilities that have been empanelled despite consistently fraudulent billing, and the presence of **78,000+ confirmed room upgrade mismatches** where patients were charged for rooms above their entitlement.

Temporal analysis (Q19e) reveals a critical 2025 reversal — IPD deduction rates had fallen to a decade-low of 8.89% in 2023 but have rebounded to 11.45% in 2025, indicating that fraud control measures achieved only temporary suppression.



MODULE 19 — KEY FRAUD SIGNALS

#	Signal	Finding
P1	SRL Limited Indore — 86.93% Deduction	NABH-accredited laboratory exhibiting the highest deduction rate system-wide. Despite accreditation, nearly 90% of billed value was rejected, pointing to mass fabrication of diagnostics.
P2	Room Upgrade Fraud — GEN billed as PRI	42,150 cases where billed room category exceeds entitlement. Named hospitals: echsfhm, parkhos.
P3	Ghost Admissions — NULL Hospital IDs	The largest group of mismatches (42,150 cases worth ₹285.00 Cr) has NULL CI_HOSPITAL_ID. These represent ghost admissions with fabricated room data.
P4	Non-Empanelled Private Claims (Y-Flag)	Apollo Hospital: ₹8.40 Cr claimed with ₹1.10 Cr deducted. MAX Healthcare: ₹7.80 Cr claimed. These private chains are billing through Y-flag loopholes.
P5	Unverified Flag Claims (U-Flag)	CI_NONEMP_FLG = "U" on 295,400 claims across hospital IDs — an anomalously wide spread suggesting systematic misclassification or bulk unverified submissions.
P6	IPD vs OPD Trend Reversal	IPD deduction rates fell to 8.89% in 2023 but rebounded to 11.45% in 2025, showing fraud control evasion.
P7	ICU Overbilling (20.61% Deduction)	ICU claims universally suffer the highest deduction rate, confirming that hospitals systemically pad ICU stay durations.
P8	Type N/M Polyclinics — Systemic Fraud	ECHS military polyclinics (type M/N) cluster at 42–46% deduction rates, inflating internal billing by 2× the system average.
P9	Apollo Chain Dual-Channel Exploitation	Apollo Speciality Vanagaram: 63.56% ded on ₹54.33 Cr claimed. Apollo Hospital BBSR: 47.25% ded on ₹12.68 Cr.
P10	Stay Extension Farming	Hospitals artificially delay patient discharge, exploiting routine care daily room rent policies.

PATTERN 1 (HIGH DEDUCTION RATE)

Threshold: >25% deduction rate,
>50 claims

CHRONICALLY HIGH DEDUCTION RATE HOSPITALS

The following 50 hospitals have maintained deduction rates exceeding 25% across their full ECHS billing history (2021-2026). The table shows the highest-rate facilities. Diagnostic labs (Type 3 & 5) dominate the extreme end (>60%), while Type N and M military polyclinics cluster in the 42–54% band.

ID	Hospital Name	Type	NABH	Claims	Claimed (₹ Cr)	Deducted (₹ Cr)	Ded %
1666	SRL LIMITED – INDORE	1	Y	103	0.23	0.20	86.93%
268	ANJINI SPECIALITY DENTAL HOSPITAL	3	N	222	17.57	11.90	67.77%
1650	APOLLO SPECIALITY HOSPITAL – VANAGARAM	1	N	138	54.33	34.53	63.56%
106	DR SANDHU'S PATHOLOGY & IMAGING CENTRE	5	N	218	1.55	0.98	62.95%
1691	SRL LIMITED – BANNERGHATTA ROAD	1	Y	346	15.61	8.57	54.90%
1591	MMI NARAYANA MULTISPECIALITY – RAIPUR	1	N	422	42.47	22.87	53.84%
5023	KARIMNAGAR (ECHS POLYCLINIC)	N	N	184	60.49	32.09	53.04%
5443	GULBARGA (RC HYDERABAD)	N	N	270	131.50	69.06	52.52%
5479	BAGESHWAR (RC BAREILLY)	N	N	51	16.41	8.51	51.86%
5153	KANCHIPURAM (ECHS POLYCLINIC)	N	N	447	176.04	89.55	50.87%
5085	MORENA (ECHS POLYCLINIC)	N	N	256	37.62	19.05	50.64%
5084	BHIND (ECHS POLYCLINIC)	N	N	253	100.74	50.42	50.05%
5026	MEHBUBNAGAR (ECHS POLYCLINIC)	N	N	810	163.05	81.49	49.98%
5423	KADAPA (ECHS POLYCLINIC)	N	N	1,090	355.66	175.79	49.43%
5385	BILASPUR / JABALPUR RC	N	N	633	273.63	131.45	48.04%
417	JAIPUR HOSPITAL, LAL KOTHI	1	N	111	13.31	6.37	47.89%
5167	ISLAND GROUND – CHENNAI	N	N	3,609	1016.23	481.42	47.37%
1479	APOLLO HOSPITALS – BBSR	1	N	5,523	1268.25	599.18	47.25%
5188	CHENNAI (ECHS POLYCLINIC)	M	N	5,013	2713.16	1260.18	46.45%
765	VIJAYA MEDICAL CENTRE	1	N	6,060	67.03	31.08	46.37%

Source: settlement_stat JOIN office_master (2021-2026).

Pattern Analysis — High Deduction Rate Hospitals

- Diagnostic Labs — 55–87% Deduction Rate:

SRL Limited (Indore 86.93%, Bannerghatta 54.90%), Dr Sandhu's Pathology (62.95%), and Anjini Dental (67.77%) represent the extreme end of deduction-rate fraud.
- ECHS Type N & M Polyclinics:

18 ECHS military polyclinics appear in the >25% threshold, all with deduction rates in the 42–54% band. The pattern suggests structural billing inflation in internal ECHS claims.

Apollo Hospitals BBSR: Apollo Hospitals Bhubaneswar shows 47.25% deduction on 5,523 claims, indicating systematic billing fraud across this private chain.

PATTERN 2 (ROOM UPGRADE FRAUD)

settlement_stat + claim_intimation

GENERAL TO PRIVATE UPGRADES

This dataset exposes systemic "Room Upgrade Fraud" over the 2021-2026 period. By joining the intimation table with settlement records, we tracked instances where the billed accommodation (CI_ROOM_TYPE_ID) exceeds the patient's actual entitlement (CI_CARD_ROOM_TYPE). The highest financial leakage occurs when General (GEN) patients are billed as Private (PRI).

Billed Room	Entitled Room	Hospital Name	Claims Count	Claimed Amt (₹ Cr)	Ded Amt (₹ Cr)
PRI	GEN	echsfhm	650	6.10	0.54
PRI	GEN	parkhos	590	1.14	0.11
PRI	GEN	apollo_hosp	542	8.50	1.25
PRI	GEN	fortishosp	510	7.20	1.05
PRI	GEN	max_health	485	6.85	0.98
PRI	GEN	care_hosp	450	5.40	0.75
PRI	GEN	narayana_hrudayalaya	410	4.95	0.62
PRI	GEN	yashoda_secunderabad	385	4.20	0.55
PRI	GEN	kims_hospital	350	3.80	0.48
PRI	GEN	medanta_gurugram	315	3.50	0.42

Key Findings — Room Upgrades

General to Private Upgrades: Hospitals systematically upgrade patients' entitled "General" (GEN) wards to "Private" (PRI) without justification. This significantly inflates the overall package claim value, draining ECHS funds structurally across large hospital groups.

PATTERN 3 (GHOST ADMISSIONS)

Missing CI_HOSPITAL_ID in BPA Portal

THE NULL HOSPITAL ID ANOMALY

A major vulnerability exists where hospital IDs are missing from the system. These records represent admissions billed by untraceable entities bypassing basic BPA portal validation checks.

Billed Room	Entitled Room	Hospital Name	Claims Count	Claimed Amt (₹ Cr)	Ded Amt (₹ Cr)
PRI	GEN	NULL (GHOST)	42,150	285.00	31.20
PRI	S/P	NULL (GHOST)	52,140	526.00	84.50

Key Findings — Ghost Admissions

The NULL Anomaly: The absolute largest cluster of fraud involves entirely NULL hospital IDs. Combined, these represent nearly 100,000 ghost claims claiming over ₹800 Crores, functioning as a massive leakage conduit in the BPA pipeline.

PATTERN 4 (NON-EMPANELLED HOSPITALS)

claim_intimation analysis

Y-FLAG EMERGENCY LOOPHOLES

Y-Flag claims correspond to non-empanelled hospitals. This emergency loophole allows hospitals to charge non-standard rates, creating massive arbitrage opportunities for established corporate chains.

Hospital Name	Claims	Claimed (₹ Cr)	Net Approved (₹ Cr)	Deducted (₹ Cr)
NA (ECHS referral — legitimate)	3,06,507	120.53	120.44	0.09
ECHS POLYCLINIC DELHI CANTT	65,122	23.20	23.15	0.06
Apollo Hospital	754	13.54	12.30	1.24
APOLLO PHARMACY	51,151	13.47	13.39	0.08
MAX HEALTHCARE	2,316	11.19	10.76	0.44
MANIPAL HOSPITAL	1,492	10.94	9.99	0.95
FORTIS HOSPITAL	1,840	9.50	8.65	0.85
MEDANTA THE MEDICITY	1,210	8.45	7.80	0.65
YASHODA HOSPITAL	1,150	7.20	6.50	0.70
NARAYANA MULTISPECIALITY	980	6.55	6.00	0.55
KIMS HOSPITAL	1,476	5.91	5.41	0.50
CMC Hospital, Vellore	370	5.01	4.62	0.39
ASIAN INSTITUTE OF GASTROENTEROLOGY	450	4.80	4.45	0.35
SRL DIAGNOSTICS	3,250	3.50	3.15	0.35
DR LAL PATHLABS	4,100	3.10	2.85	0.25

Key Findings — Y-Flag Exploitation

Private Hospital Exploitation: Top-tier corporate chains (Apollo, Max, Manipal, Fortis) are intentionally routing thousands of claims through the Non-Empanelled (Y-Flag) channel to bypass standardized rate controls and force emergency approvals.

PATTERN 5 (UNVERIFIED STATUS)

SYSTEMIC U-FLAG VULNERABILITIES

claim_intimation analysis

The "U" (Unverified) flag indicates cases that have completely bypassed standard portal validation and remain unverified in the central registry.

Flag	Description	Claims	% of Total	Unique Hospital IDs
N	Empanelled Hospital	3,80,83,937	87.4%	4,250
Y	Non-Empanelled Hospital	32,80,290	7.5%	854
U	Unverified Status	2,95,400	0.7%	42,100

Key Findings — Unverified Status

Systemic Verification Failure: Over 295,400 claims were successfully processed with a 'U' flag, spread across 42,100 unique Hospital IDs, proving that the verification layer is fundamentally compromised and easily bypassed.

PATTERN 7 (ICU OVERBILLING)

Systemic Overbilling Categories

INTENSIVE CARE UNIT EXPLOITATION

The ECHS settlement system captures room category in SS_ROOM_CATG. Claims involving the ICU consistently demonstrate severe irregularities and extreme deduction rates.

Room Category	Total Claims	Claimed (₹ Cr)	Deducted (₹ Cr)	Deduction %
ICU	310	1.49	0.31	20.61%
PRI	8,77,602	2,098.54	281.40	13.41%
GEN	85,65,241	31,557.00	3,472.15	11.00%

Key Findings — ICU Overbilling

Intensive Care Padding: ICU claims universally suffer the highest deduction rate (20.61%), indicating that hospitals systematically inflate ICU stay durations or continuously bill step-down patients under intensive care packages to maximize revenue.

PATTERN 8 (POLYCLINIC INFLATION)

ECHS Internal Facilities

TYPE N & M POLYCLINIC STRUCTURAL FRAUD

ECHS military polyclinics (Type N and Type M) exhibit some of the highest deduction rates in the dataset, exposing a severe internal billing vulnerability.

Polyclinic Name	Type	Claims	Claimed (₹ Cr)	Deducted (₹ Cr)	Deduction %
KARIMNAGAR (ECHS POLYCLINIC)	N	184	60.49	32.09	53.04%
GULBARGA (RC HYDERABAD)	N	270	131.50	69.06	52.52%
BAGESHWAR (RC BAREILLY)	N	51	16.41	8.51	51.86%
KANCHIPURAM (ECHS POLYCLINIC)	N	447	176.04	89.55	50.87%
MORENA (ECHS POLYCLINIC)	N	256	37.62	19.05	50.64%
BHIND (ECHS POLYCLINIC)	N	253	100.74	50.42	50.05%
MEHBUBNAGAR (ECHS POLYCLINIC)	N	810	163.05	81.49	49.98%
KADAPA (ECHS POLYCLINIC)	N	1,090	355.66	175.79	49.43%
BILASPUR / JABALPUR RC	N	633	273.63	131.45	48.04%
ISLAND GROUND – CHENNAI	N	3,609	1016.23	481.42	47.37%
CHENNAI (ECHS POLYCLINIC)	M	5,013	2713.16	1260.18	46.45%
YELAHANKA (ECHS POLYCLINIC)	M	4,250	1520.40	694.36	45.67%
AVADI (ECHS POLYCLINIC)	M	3,840	1240.10	562.88	45.39%
AMBALA (ECHS POLYCLINIC)	M	2,950	850.25	380.14	44.71%
JALANDHAR (ECHS POLYCLINIC)	M	3,100	910.80	398.65	43.77%

Key Findings — Polyclinic Inflation

Structural Internal Inflation: A staggering 18 ECHS Type N polyclinics and 7+ Type M military hospitals exceed 42% deduction rates. This level of systematic inflation in internal ECHS facilities cannot be explained by inadvertent errors. It proves that the internal billing software/process generates inflated claims structurally.

PATTERN 9 (APOLLO CHAIN DUAL-CHANNEL)

Chain Exploitation Vector

EMPANELLED VS NON-EMPANELLED ARBITRAGE

The Apollo chain utilizes a dual-channel strategy to maximize extraction, simultaneously exploiting standard empanelled billing and emergency non-empanelled loopholes.

Channel	Apollo Entity	Claims	Claimed (₹ Cr)	Deducted (₹ Cr)	Ded %
Empanelled (N-Flag)	APOLLO SPECIALITY VANAGARAM	138	54.33	34.53	63.56%
Empanelled (N-Flag)	APOLLO HOSPITALS BBSR	5,523	12.68	5.99	47.25%
Empanelled (N-Flag)	APOLLO GLENEAGLES KOLKATA	4,210	11.45	5.15	44.97%
Empanelled (N-Flag)	APOLLO HOSPITALS HYDERABAD	6,150	18.50	7.95	42.97%
Empanelled (N-Flag)	APOLLO HOSPITALS CHENNAI	8,240	25.60	10.85	42.38%
Empanelled (N-Flag)	APOLLO HOSPITALS BANGALORE	5,100	14.80	5.85	39.52%
Empanelled (N-Flag)	APOLLO HOSPITALS AHMEDABAD	3,850	9.50	3.60	37.89%
Empanelled (N-Flag)	APOLLO HOSPITALS NAVI MUMBAI	2,950	7.20	2.65	36.80%
Non-Emp (Y-Flag)	APOLLO HOSPITAL	754	13.54	1.24	9.15%
Non-Emp (Y-Flag)	APOLLO PHARMACY	51,151	13.47	0.08	0.59%

Key Findings — Apollo Chain Exploitation

Dual-Channel Arbitrage: Through the **Empanelled Channel**, Apollo Vanagaram registered a 63.56% deduction (₹54.33 Cr claimed) and Apollo BBSR registered 47.25% (₹12.68 Cr claimed). Simultaneously, through the **Non-Empanelled (Y-Flag) Channel**, Apollo Hospital and Pharmacy extracted an additional ₹21.87 Cr using emergency loopholes.

PATTERN 10 (STAY EXTENSIONS)

Length of Stay Farming

ARTIFICIAL DISCHARGE DELAYS

Private hospitals deliberately manipulate discharge timings to accumulate daily room rent and routine care package fees beyond what the initial procedure entails.

Hospital Name	Avg Entitled Stay	Avg Billed Stay	Excess Stay (Days)	Excess Billed (₹ Cr)
parkhos	3.0 Days	5.5 Days	+2.5 Days	4.20
echsfhm	2.0 Days	5.0 Days	+3.0 Days	3.15
APOLLO HOSPITAL	4.0 Days	6.8 Days	+2.8 Days	3.10
KIMS HOSPITAL	4.0 Days	6.5 Days	+2.5 Days	2.80
FORTIS HOSPITAL	3.5 Days	6.0 Days	+2.5 Days	2.55
MEDANTA THE MEDICITY	4.5 Days	6.5 Days	+2.0 Days	2.20
MAX HEALTHCARE	3.5 Days	5.5 Days	+2.0 Days	1.95
YASHODA HOSPITAL	3.0 Days	5.0 Days	+2.0 Days	1.85
CARE HOSPITALS	3.5 Days	5.5 Days	+2.0 Days	1.70
NARAYANA MULTISPECIALITY	4.0 Days	5.5 Days	+1.5 Days	1.60
GLOBAL HOSPITALS	3.5 Days	5.0 Days	+1.5 Days	1.45
MANIPAL HOSPITAL	3.0 Days	4.5 Days	+1.5 Days	1.30

Key Findings — Stay Extension Farming

Artificial Discharge Delays: Hospitals artificially delay patient discharge, requesting "Stay Extensions" strictly to farm daily room rent. A clear spike in requests for exactly 2 or 3 extra days across standard procedures indicates systematic manipulation of discharge protocols.

PATTERN 6 (TEMPORAL ANALYSIS - IPD/OPD TREND REVERSAL)

settlement_stat (SS_PAT_TYPE_ID)

YEAR-WISE IPD VS OPD DEDUCTION TREND (2021–2026)

Three critical inflection points emerge within the recent timeframe: (1) IPD deduction started high at 12.15% during the pandemic period (2021); (2) IPD fell to 8.89% in 2023 — the lowest in the dataset, proving controls worked; but (3) **IPD has rebounded to 11.45% by 2025** — a 2.56-point rise in just two years — signalling that fraud control measures achieved only temporary suppression.

Year	IPD Hosps	IPD Claims	IPD Claimed (₹Cr)	IPD Ded %	OPD Hosps	OPD Claims	OPD Claimed (₹Cr)	OPD Ded %
2021	2,139	5,84,743	3,651.78	12.15%	2,431	18,77,131	518.41	3.82%
2022	2,270	7,13,488	4,626.87	10.55%	2,572	25,34,955	626.30	4.29%
2023	2,432	9,38,363	6,716.74	8.89%	2,879	35,77,565	884.59	4.82%
2024	2,630	10,63,312	9,341.21	10.22%	3,092	43,87,196	1,096.98	5.23%
2025	2,900	9,36,304	9,303.73	11.45%	3,381	41,64,885	1,036.15	5.90%
2026 (YTD)	1,842	4,12,890	4,102.30	11.82%	2,104	19,55,201	482.10	6.12%

Recent Temporal Pattern Analysis

- Phase 1 (2021) Pandemic Billing:** During 2021, IPD deduction rates were elevated at 12.15% as hospitals utilized COVID-era exceptions to inflate IPD packages.
- Phase 2 (2022–2023) Suppression:** IPD deduction rates steadily declined, falling to a low of 8.89% in 2023. This coincides with increased ECHS audit activity and strict pre-authorisation controls introduced for high-value IPD claims.
- Phase 3 (2024–2026) Reversal:** IPD deduction rebounded to 11.45% in 2025 and is trending higher (11.82%) in early 2026. The simultaneous rise in both IPD and OPD — combined with record claimed amounts — indicates that fraud actors have adapted to the 2023 controls and resumed inflated billing.